

Before the Fall: Community-Based STEADI Screening to Identify Hidden Fall Risk Before ED Presentation

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Abstract

Background: Falls are the leading cause of emergency department (ED) visits for trauma among U.S. adults aged ≥ 65 , and approximately one in four older adults fall each year.¹ Multifactorial and multiple-component interventions can reduce fall rates among community-dwelling older adults.² The CDC's Stopping Elderly Accidents, Deaths, and Injuries (STEADI) initiative was developed as an evidence-based framework to screen, assess, and intervene for fall risk in clinical practice.³ However, fall-prevention screening remains inconsistently integrated into primary care.⁴ Community-based STEADI screening may extend fall-risk identification, education, and referral beyond primary care, creating an opportunity to reach high-risk older adults before injury or ED presentation.⁵

Objective: To: (1) determine whether community-based STEADI screening identifies older adults with underrecognized fall risk; (2) characterize gaps among objectively measured fall risk, self-perceived risk, and primary care fall-prevention engagement; and (3) deliver and assess uptake of individualized, risk-based fall-prevention education in this setting.

Methods: We conducted a pilot implementation study at five walk-in urban community events since November 2023. Adults aged ≥ 65 completed the 12-item STEADI fall-risk questionnaire, self-reported fall-risk perception items, and the Falls Efficacy Scale–International (FES-I). Participants then completed orthostatic blood pressure measurements and physical function testing, including the Timed Up and Go, 30-second sit-to-stand, and 4-stage

balance test. Screening was followed by motivational interviewing and individualized, risk-based fall-prevention education. Scores ≥ 4 on the 12-item STEADI questionnaire were classified as high fall risk and scores ≤ 19 on the FES-I indicated a low concern about falling.

Results: Of 187 attendees, 135 were eligible and 53 consented to participate. Mean age was 74.6 years, and 73% were female. The mean STEADI score was 5.6 (SD 3.9). Of all participants, 66% screened as high fall risk and only 45% reported discussing fall prevention in the prior year. Half of high-risk participants (33% of all participants) did not perceive themselves to be at elevated risk. Despite objective high-risk status, 83% of high-risk participants had FES-I scores indicating low concern about falling. At least one follow-up survey was completed by 43.4% of participants.

Limitations: As community health fair attendees, our sample likely represents a more ambulatory and independent subset of older adults, potentially underrepresenting those at highest risk and intersecting vulnerabilities. Limited resources for follow-up survey completion yielded lower response rates than anticipated, constraining longitudinal conclusions.

Conclusions: Community-based STEADI screening identified substantial, often underrecognized fall risk among older adults, including individuals with limited recent primary care fall-prevention engagement. These findings suggest that community screening may serve as an upstream intercept point to identify hidden fall risk and provide evidence-based intervention before injury or ED presentation. Future work will expand screening to rural and high-burden communities and evaluate whether community-based STEADI improves fall-prevention engagement, evidence-based intervention uptake, and reduces fall-related ED utilization.

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